

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 7

New Case: Rohan (26 y/o) – Contamination OCD + Checking

Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session #:	7 of 18–20	Focus:	Continuing ERP + Addressing Difficulties

PRE-SESSION THERAPIST NOTES

Client arrived looking more confident but slightly frustrated. He practiced the door handle exposure at home 3 times. He reported good habituation on two attempts but struggled significantly on one day when anxiety was higher and ended up washing more than planned. He continues to show good engagement with labeling and monitoring. Today's plan: Review home ERP practice in detail (including the difficult attempt), continue ERP with the next hierarchy item if appropriate, address any difficulties or resistance, reinforce progress, and assign new homework. Maintain supportive and non-judgmental approach.

SESSION START – HOMEWORK REVIEW

Therapist: Good morning, Rohan. How has your week been? I'm interested to hear how the door handle practice at home went.

Client: (speaking with a mix of pride and frustration) It was mixed. I practiced touching the door handle and delaying washing three times. Two times went quite well — anxiety went up to around 40–45 and then came down over 6–8 minutes. I was able to wait without washing excessively. But on one day, after touching the handle, the anxiety spiked very high and the urge felt overwhelming. I ended up washing more than I wanted to. I felt disappointed in myself afterward.

Therapist: Thank you for being honest about both the successes and the difficult day. It's very normal for progress to be uneven — some days are easier than others, especially when anxiety is higher or when other stressors are present. The fact that two out of three attempts went well is still meaningful progress. What do you think made the difficult day harder?

Client: I think I was already stressed from work that day. My mind was racing with other things, so when the contamination thought came, it felt much stronger than usual. I couldn't focus on labeling or breathing properly.

Therapist: That's a very important observation. When we're already stressed or mentally fatigued, OCD thoughts can feel louder and harder to manage. This doesn't mean you failed — it means external factors can influence how strong the OCD feels on any given day. We can use this understanding to be more compassionate with ourselves on difficult days and adjust expectations.

CONTINUING ERP – NEXT HIERARCHY ITEM

Therapist: Given that you had both successes and a difficult day, I'd like to suggest we continue with the next item on your hierarchy today in session — touching a lift button (using tissue first) and then delaying washing for a longer period. We can practice it here first. How does that feel?

Client: I think I can try it. I'm a bit nervous after the difficult day at home, but practicing here with your support might help.

Therapist: That's completely understandable. Let's do it step by step. I'll ask you to touch the lift button (using a tissue first, as per your hierarchy). Then we'll stay with whatever thoughts and anxiety come up, using labeling and breathing. We'll aim to delay washing for at least 8–10 minutes. Ready when you are.

(In-session exposure conducted: Rohan touched the lift button using tissue. Initial anxiety rose to around 50/100. He practiced labeling and breathing. After 9 minutes, anxiety decreased to around 22/100. He reported that the urge weakened significantly over time, though it was stronger than the previous exposure. Post-exposure processing done.)

Therapist: How was that experience for you?

Client: It was harder than the door handle one, but I could handle it. The anxiety went up more, but it still came down after some time. I feel a bit tired now, but also relieved that I was able to do it without washing immediately.

Therapist: That's a meaningful step forward. You tolerated higher anxiety and still experienced habituation. This builds your confidence that you can handle more difficult situations over time. How are you feeling about practicing this one at home?

Client: I think I can try it 2–3 times this week. I'm still nervous, but today's practice helped.

HOMEWORK ASSIGNMENT

Therapist: For this week:

1. Practice touching a lift button (using tissue) and delaying washing for 8–10 minutes (2–3 times)
2. Continue practicing the door handle exposure at home
3. Continue labeling and monitoring

Client: I'll try the lift button one. I feel more prepared after today's session.

Therapist: Excellent. We'll review how it went next session.

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed both your successes and the difficult day, did the next hierarchy item in session, and saw good habituation again. How are you feeling about today's session?

Client: I feel okay. It was helpful to talk about the difficult day without feeling judged. Doing the exposure here gave me more confidence. I'm still scared of some higher items, but I think I can keep going.

Therapist: That's completely fair. Any questions before we finish?

Client: Not really.

POST-SESSION THERAPIST NOTES

- Client showed continued progress in ERP despite one difficult day at home. Successfully completed third hierarchy item in session with good habituation.
- Client demonstrated good insight into how external stress can amplify OCD symptoms — important for self-compassion and realistic expectations.
- Motivation remains strong. Client is gradually building tolerance to higher anxiety levels through repeated exposure practice.
- Next session: Review home ERP practice, continue ERP with next hierarchy item if ready, reinforce coping skills and self-compassion, address any emerging resistance or avoidance.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.